A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 May 2026

Dr Andrew Sampson
Doctors Of Northcote

Dear Dr Sampson,

Re: Mr Samuel Lingham
DOB: 11/11/1989 Phone: 0427 020 486
Address: 30 Albert Street, ABBOTSFORD, VIC, 3067
Email: samlingham@hotmail.com

Summary

- **Excellent clinical response to nortriptyline with resolution of nausea and vomiting.**
- **Improved bowel frequency, though consistency remains variable.**
- **Plan to wean medication once current work-related stressors have stabilised.**
- **Provided scripts for 25 mg and 10 mg nortriptyline tablets.**
- **No action required by you.**

I had the pleasure of reviewing Samuel in the clinic today. He has been taking nortriptyline for approximately nine months, having commenced therapy last August. I am pleased to report that he has had a transformative response to this treatment.

Prior to starting nortriptyline, Samuel was experiencing significant nausea, vomiting, and a heavy epigastric sensation associated with frequent burping. He now reports that these symptoms have entirely resolved. His bowel habit has also improved; whereas he was previously opening his bowels twice daily, he now averages once per day. While the consistency remains somewhat inconsistent and he still experiences occasional diarrhoea—often associated with specific food triggers—his overall quality of life is much improved. Indeed, he feels his symptoms are sufficiently controlled that he has been able to be more liberal with his intake of caffeine and alcohol.

He has experienced some expected side effects, including dry mouth and increased appetite. He also notes significant morning drowsiness, which is likely a result of taking the medication immediately before bed.

Impression and Plan

Samuel is understandably keen to eventually trial a period off the medication to establish his clinical baseline. However, he is currently navigating a period of significant work-related stress. We discussed the fact that about a third of patients remain asymptomatic after weaning, a third have mild symptoms that do not require treatment, and a third experience a full relapse. Given that stress is a known perpetuating factor for his symptoms, I have advised him to maintain his current dose until his workload becomes more manageable, likely in one to two months.

To simplify his regimen, I have suggested he move to a single 25 mg tablet of nortriptyline rather than his current 20 mg dose (two 10 mg tablets), as most patients do not notice a clinical difference between these

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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doses. I have provided him with scripts for both 25 mg tablets for maintenance and 10 mg tablets to facilitate a weaning process when he feels the time is right. Our plan is for him to reduce the dose to 10 mg for one month before ceasing.

He will contact me for a review on an as-needed basis.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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